

FOCUS HEALTH

Gastric Cancer Awareness

Dr. Pratik Patel
Medical Oncologist
Pune

What is Stomach or Gastric Cancer?

Stomach or gastric cancer, is 5th most common cause of cancer mortality in the world. 90-95% cases develop from gland cells in mucosa of stomach. The stomach wall has 5 layers, and understanding how deeply tumor invades guides staging, treatment choices.

Stomach cancer mostly affects older people, average age at diagnosis around 68 years, and men having higher lifetime risk. In the United States, declining Helicobacter pylori infection and lower consumption of salted and smoked foods have reduced incidence. However, gastric cancer remains significantly more common in East Asia, where population-based screening programs have enabled detection at an early stage.

Stomach cancer can be hard to spot early as symptoms include poor appetite or weight loss without trying, abdominal pain, bloating, feeling of fullness after small meals, indigestion, nausea, vomiting, brown or black stools. While many of these symptoms have non-cancer causes, persistent issues need medical evaluation to identify and manage.

Diagnosis of Stomach or Gastric Cancer

Diagnosis relies on clinical assessment and targeted testing. Upper endoscopy is done to visualize esophagus, stomach, and first part of small intestine, with biopsy from abnormal areas for cancer detection. For cancers deeper in the stomach wall, endoscopic ultrasound can be done. If cancer is identified, additional laboratory studies may characterize tumor biology, including HER2, PD-L1, MSI-H, and mismatch repair (dMMR) testing. Imaging with CT evaluates local extent and spread, including involvement of the liver and nearby lymph nodes. PET scanning can help assess distant metastases across the body. If cancer is identified without distant spread, diagnostic laparoscopy may be performed to confirm staging before definitive surgery. Staging helps determine how serious the cancer is and how best to treat it. The American Joint Committee on Cancer (AJCC) TNM staging system is most commonly used for gastric cancer. T describes primary tumor extent, N indicates regional lymph nodes involvement, and M denotes distant metastasis, with stages ranging from 0 to IV.

Treatment of Stomach or Gastric Cancer

Treatment depends on stage, location, and overall health. Endoscopic mucosal resection (EMR) and endoscopic submucosal dissection (ESD) are procedures used to treat early-stage cancers. When endoscopic therapy is not suitable, surgical options include partial (subtotal) gastrectomy for tumors limited to the upper or lower stomach, sometimes involving part of the esophagus (proximal gastrectomy) or the first segment of the small intestine (distal gastrectomy) followed by reconstruction. In total gastrectomy, surgeon removes the entire stomach and may extend to adjacent organs. In all surgical approaches, removal of nearby lymph nodes (lymphadenectomy) is performed because the number of lymph nodes removed is linked to surgical success.

Chemotherapy can be given before or after surgery and may help can shrink tumors, improve resectability, and lower relapse rates. For stomach cancer, doctors often prefer combinations of 2 or 3 drugs to get best of the results and survival outcomes.

Newer treatments like targeted drugs and immunotherapy have expanded options for advanced stages. Targeted drugs target molecular changes and can be given either alone or along with chemotherapy. In some people with stomach cancer, the cancer cells have too much of a growth promoting protein called HER2 on their surface. Adding anti-HER2 targeted therapy to chemotherapy can improve outcomes compared with chemotherapy alone. Tumors make new blood vessels to get blood and nutrients for growth with VEGF protein. Anti-VEGF monoclonal antibodies bind to VEGF receptors, thus slowing the growth of advanced stomach cancers. Immune system uses checkpoint proteins to prevent itself from attacking normal cells in the body. Cancer cells sometimes use these to avoid being attacked by immune cells. Immunotherapy helps person's own immune system destroy cancer cells more effectively. These immunotherapy drugs boost the immune response by reactivating the body's immune system to fight against cancer cells.

Care does not end with treatment completion. Most doctors recommend follow-up visits every 3 to 6 months for the first few years to monitor recovery and toxicity. Nutrition optimization and psychosocial support remains integral, helping patients make informed decisions throughout their cancer journey.

References:

1. *American Cancer Society. About Stomach Cancer*
2. *American Cancer Society. Stomach Cancer Causes, Risk Factors, and Prevention*
3. *American Cancer Society. Stomach Cancer Early Detection, Diagnosis, and Staging*
4. *American Cancer Society. After Stomach Cancer Treatment*
5. *National Cancer Institute. Stomach (Gastric) Cancer Screening (PDQ)- Health Professional Version*
6. *National Cancer Institute. What is stomach cancer?*
7. *National Cancer Institute. Stomach Cancer Symptoms*

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